Explaining the failure of global health governance during COVID-19

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Global health governance (GHG) was one of the COVID-19 pandemic's earliest victims. The World Health Organization (WHO), the international agency responsible for managing pandemic disease, was prominent during the initial outbreak in Wuhan, but was quickly marginalized amid acrimony between the United States and China. The virtual 'collapse of global cooperation' followed, as governments pursued ad hoc, uncoordinated and even competitive measures to control COVID-19.[1] The WHO's 2005 International Health Regulations (IHR) aimed to prevent precisely this outcome but, facing their first truly global challenge, they failed, becoming irrelevant almost immediately. States resorted to measures such as lockdowns that had never been part of WHO (or their own) pandemic planning, while ignoring WHO guidance against travel bans and border closures, often failing even to notify the WHO of deviations from the IHR, as the regulations require. Although the WHO facilitated some data-sharing and technical cooperation, its scientific guidance was often highly controversial and its major initiatives, on access to diagnostics, therapeutics and vaccines, were crippled by 'vaccine nationalism' and wealthy states' inadequate commitments.[2]

Why did GHG fail? Two broad explanations have emerged. The first foregrounds the conduct of powerful states. US President Trump and others blamed the WHO's subservience to China for its sluggishness in declaring a Public Health Emergency of International Concern (PHEIC), damaging its credibility.[3] Others dismissed this

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1 Allyn L. Taylor and Roojin Habibi, 'The collapse of global cooperation under the WHO International Health Regulations at the outset of COVID-19: sculpting the future of global health governance', *American Society of International Law Insights* 24: 15, 2020, https://www.asil.org/insights/volume/24/issue/15/collapse-global-cooperation-under-who-international-health-regulations. (Unless otherwise noted at point of citation, all URLs cited in this article were accessible on 3 Oct. 2022.)

2 Matthew Kavanagh, Renu Singh and Mara Pillinger, 'Playing politics: the World Health Organization's response to COVID-19', in Scott L. Greer, Andrea J. King, André Peralta-Santos and Elize Massard da Fonseca, eds, *Coronavirus politics: the comparative politics and policy of COVID-19* (Ann Arbor: University of Michigan Press, 2021), pp. 34–50; Darren Lilleker and Miloš Gregor, 'World Health Organization: the challenges of global leadership', in Darren Lilleker, Ioana A. Coman, Miloš Gregor and Edoardo Novelli, eds, *Political communication and COVID-19: governance and rhetoric in times of crisis* (Abingdon: Routledge, 2021), pp. 19–33.

3 David P. Fidler, *The World Health Organization and pandemic politics*, Council on Foreign Relations—Think Global Health, 10 April 2020, https://www.thinkglobalhealth.org/article/world-health-organization-and-

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as an attempt to scapegoat the WHO for domestic failures,[4] blaming the GHG crisis instead on Trump's hostility to multilateralism and the absence of US leadership.[5] The second and more common explanation is that states' reluctance to sacrifice their sovereignty meant that the WHO had not been empowered to enforce compliance with the IHR—it 'failed by design'.[6] COVID-19 was thus a 'Westphalian disease'.[7] Many commentators now despair of the prospect of meaningful, 'globalist' change while 'statist' security agendas trump international cooperation.[8]

The problem is that, rather than evaluating GHG as it actually exists, both perspectives criticize it for not being what they would like it to be: a system in which a competent and independent WHO manages pandemics authoritatively, including by enforcing its rules in recalcitrant states. This desired state is reflected in proposals for WHO reform, which frequently demand that states give the WHO greater enforcement powers.[9] However, global governance rarely, if ever, involves empowering supranational organizations to manage issues directly, nor does it primarily work by enforcing rules *vis-à-vis* member states. This is because no international organization (IO), even if lavishly resourced and legally authorized, can manage any issue without the active collaboration of the authorities 'on the ground'. Consequently, most global governance initiatives focus not on empowering supranational authorities, but on transforming states' domestic governance in line with international regulations. IOs act as 'meta-governors', specifying priorities, targets, 'best practice' processes and policies, while promoting states' transformation to enact these domestically. To explain the collapse of GHG convincingly, we must explain the failure of *this* approach—'global governance as state transformation' (GGST)[10]—not lament states' failure to do something entirely different. From this perspective, GHG failed because states lacked the necessary capacities to manage COVID-19, not because the WHO lacked authority over them.

We argue that GGST has three interrelated weaknesses, each becoming manifest in GHG during the COVID-19 pandemic. First, GGST is inherently contradictory because, while its emergence relies on neo-liberal state transformation, these same changes also hollow out the state capacities upon which its efficacy relies. Second, this problem is compounded by GGST's depoliticized, de-democratized

pandemic-politics; Lawrence O. Gostin, 'COVID-19 reveals urgent need to strengthen the World Health Organization', *Journal of the American Medical Association* 323: 23, 2020, pp. 2361–2.

4 Sophie Harman, 'COVID-19, the UN, and dispersed global health security, *Ethics and International Affairs* 34: 3, 2020, pp. 373–8.

5 Joshua W. Busby, 'Understanding the anemic global response to COVID-19', *Journal of Health Politics, Policy and Law* 45: 6, 2020, pp. 1013–21; Ashish Jha, 'System failure: America needs a global health policy for the pandemic age', *Foreign Affairs* 100: 2, 2021, pp. 103–15.

6 Stewart Patrick, 'When the system fails: COVID-19 and the costs of global dysfunction', *Foreign Affairs* 99: 4, 2020, p. 40; also, e.g., Gostin, 'COVID-19 reveals'; Taylor and Habibi, 'The collapse'; Kavanagh et al., 'Playing politics'.

7 Fidler, *The World Health Organization*.

8 Clare Wenham, Mark Eccleston-Turner and Maike Voss, 'The futility of the pandemic treaty: caught between globalism and statism', *International Affairs* 98: 3, 2022, pp. 837–52.

9 Wenham et al., 'The futility', pp. 848–50; Lidia Kuznetsova, 'COVID-19: the world community expects the World Health Organization to play a stronger leadership and coordination role in pandemics control', *Frontiers in Public Health*, vol. 8, 2020, p. 470; Gostin, 'COVID-19 reveals'.

10 Shahar Hameiri and Lee Jones, 'Global governance as state transformation', *Political Studies* 64: 4, 2016, pp. 793–810.

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approach, which results in a gap between global governance agendas and domestic political processes, undermining implementation. Finally, domestic socio-political power relations inevitably condition the form and extent of implementation. Combined, these weaknesses make GGST regimes, such as the IHR, far feebler than they may appear on paper.

The argument proceeds in four sections. The first outlines the historical emergence of GGST and its primary features. The second discusses its weaknesses. The third section applies these insights to GHG, focusing on the WHO's shift from health-care capacity-building towards meta-governance and the limits of this approach to pandemic management, as exposed in earlier disease outbreaks. The final section demonstrates how GGST's weaknesses became manifest during COVID-19, with hollowed-out states unprepared to cope with the disease and quickly resorting to desperate ad hoc measures that departed from the IHR.

Our overall argument is that GGST, and its manifestation in GHG, was always a somewhat illusory system. Addressing global issues by issuing plans, strategies and blueprints at an international level creates a misleading impression that real action is being taken to mitigate serious threats to human well-being. The real action—or inaction—will ultimately occur at the domestic level, where (non-) implementation will ultimately depend on local socio-political power relations, despite efforts to circumvent this and empower technical experts. Consequently, to avoid similar catastrophes in future, we must pay less attention to global rules and institutions, and to the relationship between IOs and their member states, than to domestic political processes. Rebuilding state capacity, authority and accountability is essential.

The emergence of global governance as state transformation

This section sketches the emergence of GGST. Historically, it was enabled by neo-liberal state transformation, involving the dismantling of postwar developmental states and their replacement by market-facilitating 'regulatory states'. GGST then emerged through efforts to harmonize policy-making across regulatory states by developing mandates, regulations and policies at the regional or global level, which state apparatuses are then tasked with implementing domestically. These developments, summarized in Table 1, are described in the following subsections.

Table 1: Core elements of global governance as state transformation

Period	*Form of state*	*Form of international organization*
1940s to 1970s	Developmental, command-and-control nation-states	International cooperation; limited scope; supportive of national autonomy
1980s to the present	Marketizing, neo-liberal, regulatory states	Transnational policy harmonization through meta-governance; expanded scope; undermining national autonomy

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Postwar developmental states and international organization

From the end of the Second World War to the mid-1970s, most western states were characterized by hierarchical institutions that authoritatively mobilized public resources and intervened directly to address social problems. This reflected the repurposing of wartime 'command-and-control' systems for peacetime objectives, underpinned by a class compromise between capitalists and organized labour, struck in the shadow of economic depression, political extremism and war. Capitalists would grant workers a rising social wage in exchange for industrial peace. Delivering this grand bargain required state institutions capable of mediating between these social forces—corporatist forms of bargaining and industrial governance—and of managing economies to support growth, full employment, redistribution and welfare provision. This typically involved extensive public ownership and/or developmental planning and the growth of welfare states. Communist states went even further, pursuing entirely planned national economies.[11]

This state form also emerged in postcolonial societies, albeit often in a weaker form. The strongest were north-east Asia's 'developmental states' and their south-east Asian emulators, where the urgent need to counter communism, in the context of weak capitalist classes, drove state managers to assume the task of national development.[12] Most postcolonial regimes attempted something similar, albeit with far less success or coherence.[13]

The forms of international organization established after the Second World War were designed to support this domestic project—a system Ruggie famously dubbed 'embedded liberalism'.[14] The Bretton Woods institutions, for example, supported national development by funding postwar reconstruction and overseeing managed exchange rates to help governments maintain domestic policy autonomy against unregulated capital flows and market fluctuations. Domestic and international institutions were thus designed to consolidate 'the primacy of national economies, national welfare states, and national societies managed by national states concerned to unify national territories and reduce uneven development'.[15]

From 1970s crisis to global governance as state transformation

In the 1970s, major political and economic crises unravelled these arrangements, leading to significant transformations in statehood and in international organization, heralding the shift to 'global governance'. In western countries, three decades of uninterrupted and relatively equitable economic growth and rising welfare increased popular expectations of governments and employers. However,

[11] Eric J. Hobsbawm, *The age of extremes: the short twentieth century, 1914–1991* (London: Michael Joseph, 1994), ch. 9.

[12] See Toby Carroll and Darryl S. L. Jarvis, eds, *Asia after the developmental state: disembedding autonomy* (Cambridge: Cambridge University Press, 2017).

[13] Vijay Prashad, *The darker nations: a people's history of the Third World* (New York: New Press, 2007).

[14] John Gerard Ruggie, 'International regimes, transactions, and change: embedded liberalism in the postwar economic order', *International Organization* 36: 2, 1982, pp. 379–415.

[15] Bob Jessop, 'Avoiding traps, rescaling states, governing Europe', in Roger Keil and Rianne Mahon, eds, *Leviathan undone? Towards a political economy of scale* (Vancouver: University of British Columbia Press, 2009), p. 99.

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as growth and profitability fell in the 1970s, this became increasingly problematic. Trade unions contested businesses' attempts to restore profitability through price rises, demanding compensatory wage increases. Government attempts to control wages sparked industrial and political strife, which elites widely perceived as a crisis of governability resulting from an excess of democracy.[16]

The neo-liberal revolt that followed addressed this crisis by suppressing popular expectations and making states less responsive to democratic pressure. Recessionary policies created mass unemployment to terminate the prices–wages spiral, while deregulation and the liberalization of capital and trade allowed production to be offshored to lower-wage economies, weakening organized labour.[17] Corporatist institutions were dismantled and replaced with pro-market alternatives, with the state's purpose shifting from protecting domestic business and mitigating social disparities to promoting international competitiveness.[18]

Authority and power, especially over economic management, increasingly shifted to non-majoritarian institutions, such as independent central banks, while state-owned enterprises were privatized, ending partial democratic control over the economy. Unelected technocrats, regulators, quasi-autonomous non-governmental organizations, consultants and public–private partnerships acquired control over vast swathes of public life and public services.[19] This was rationalized—indeed, celebrated—as a turn to policy-making based on expertise, not narrow political considerations.[20] By dispersing the state's policy-making authority and substantive industrial and bureaucratic capacities, ruling elites deliberately withdrew from direct responsibility for, and accountability over, many areas of public policy.[21] Rather than directly intervening to achieve social goals, central government retreated to 'negative coordination', steering diverse actors in favoured directions: thus was born the 'regulatory state'.[22]

The crisis of rising expectations, and the revolt against them, also manifested itself internationally, refashioning international cooperation. From the 1960s, rising working-class demands domestically were paralleled in the international arena by increasing assertiveness on the part of Third World countries. Through the Non-Aligned Movement and other groupings, many postcolonial states pursued a 'Third World project' combining anti-imperialism, anti-racialism, respect for sovereignty, and efforts to reform national and global economic governance to overcome neo-colonialism and dependency. This peaked in the 1970s with the OPEC oil embar-

[16] Michel J. Crozier, Samuel P. Huntington and Joji Watanuki, *The crisis of democracy: report on the governability of democracies to the Trilateral Commission* (New York: New York University Press, 1975).

[17] David Harvey, *A brief history of neo-liberalism* (Oxford: Oxford University Press, 2005).

[18] Philip G. Cerny, 'Paradoxes of the competition state: the dynamics of political globalization', *Government and Opposition* 32: 2, 1997, pp. 251–74.

[19] Colin Leys, *Market-driven politics: neo-liberal democracy and the public interest* (London: Verso, 2003); Gavin Poynter, *The political economy of state intervention: conserving capital over the West's long depression* (Abingdon: Routledge, 2020).

[20] Frank Vibert, *The rise of the unelected: democracy and the new separation of powers* (Cambridge: Cambridge University Press, 2007).

[21] Matthew Flinders and Jim Buller, 'Depoliticisation: principles, tactics and tools', *British Politics* 1: 3, 2006, pp. 293–318.

[22] Kanishka Jayasuriya, 'Globalisation and the changing architecture of the state: regulatory state and the politics of negative coordination', *Journal of European Public Policy* 8: 1, 2001, pp. 101–23.

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goes and calls for a New International Economic Order (NIEO), a radical demand to redistribute power and wealth amounting to 'socialism among states'.[23] The NIEO's key features included assertions of absolute sovereignty over domestic natural resources; the creation of state-managed commodity cartels; tougher regulation for transnational corporations; North–South technology transfers; trade concessions; and debt relief.[24] Proponents advanced these demands through majoritarian IOs (including, as we shall see, the World Health Assembly), which they also sought to strengthen.[25] In 1974, the UN General Assembly adopted the Declaration for the Establishment of the NIEO, followed by meetings between the Group of 77 and UN Conference on Trade and Development to develop a joint action plan. These events reinforced Northern elites' perception of a global 'crisis of governability' arising from a 'revolt of the subalterns' at home and abroad.[26]

In the 1980s, the neo-liberal revolt was extended to the Third World and the majoritarian IOs through which they had pressed their demands. The early 1980s debt crisis crippled many developing countries. The International Monetary Fund (IMF) and World Bank imposed market-confirming structural adjustment upon them, essentially terminating the Third World project. Subsequent reform programmes, entailing extensive privatization and deregulation, converted developmental states into 'governance states' modelled on those in the North.[27] Just as they sought to de-democratize governance at home, Northern states also weakened majoritarian institutions globally, defunding and marginalizing them, while empowering new mechanisms that could be dominated by the global North. Consequently, rather than supporting national policy autonomy and development, international institutions (including, as we shall see, the WHO) increasingly moved towards facilitating market-supporting state transformation.

Neo-liberalism's triumph, reinforced by the post-Cold War dearth of ideological contestation, facilitated the shift from 'international organization' to 'global governance', specifically GGST. Issues that had often been hotly contested domestically were depoliticized, with decision-making shifting first from elections and parliaments to technical, non-majoritarian bodies such as independent central banks, regulators and standards bodies, and second from the national to the international level. These moves were mutually reinforcing. Increasingly, elites and commentators insisted that states could not solve their most serious problems alone but must collaborate on everything from environmental degradation to terrorist and pandemic threats.[28] However, by the 2000s, traditional forms of international cooperation were often seen as 'gridlocked' and 'failing' due to interstate tensions, especially resistance from 'Westphalian' rising powers.[29] The solution was seen

[23] Nils Gilman, 'The New International Economic Order: a reintroduction', *Humanity* 6: 1, 2015, p. 4.
[24] Gilman, 'The New International Economic Order', p. 3.
[25] Gilman, 'The New International Economic Order', p. 5; Prashad, *The darker nations*.
[26] Gilman, 'The New International Economic Order', p. 7.
[27] Graham Harrison, *The World Bank and Africa: the construction of governance states* (Abingdon: Routledge, 2004).
[28] See Shahar Hameiri and Lee Jones, *Governing borderless threats: non-traditional security and the politics of state transformation* (Cambridge: Cambridge University Press, 2015).
[29] Thomas Hale, David Held and Kevin Young, *Gridlock: why global cooperation is failing when we need it most* (Cambridge: Polity, 2013).

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as depoliticizing issues internationally, empowering cool-headed technocrats to develop shared rules and regulations behind closed doors,[30] then implementing these by refashioning their states' domestic governance. This new approach to international cooperation—GGST—thus seeks to govern global and/or transnational issues by transforming ostensibly domestic agencies 'in ways favoured by international actors in order to enact international disciplines on other parts of their states and/or societies'.[31]

Global governance as state transformation: key features

GGST manifests itself in three often interrelated ways, all relevant to the IHR case. The first is the transboundary networking of state apparatuses. Neo-liberal state transformation has dispersed state authority to diverse, fragmented actors with enhanced autonomy from domestic democratic constraints, allowing many to engage, and increasingly develop joint policies with, their overseas counterparts.[32] However, this rarely involves investing IOs with supranational policy-making and enforcement capacities. Rather, these networks develop rules, standards, guidelines and other forms of 'soft' international law, which national agencies are then directed to embed domestically. Implementation is typically monitored and encouraged through benchmarking against internationally agreed standards, rather than coercion. A notable example is the Financial Action Task Force (FATF), a network of financial regulators that combats money laundering and terrorist financing by developing detailed legal, policy and institutional templates, then monitoring their domestic implementation through regular peer review.[33]

Second, GGST manifests itself through powerful states' ostensibly domestic institutions acquiring international functions. Sometimes collaborating with IOs, NGOs and the private sector, these functional agencies often engage directly with their counterparts in states where transnational problems are seen to originate, promoting their transformation to address these issues. This involves developing and funding projects to build the desired capacities and processes on the ground, backed by international monitoring. For example, the US Coast Guard's (USCG) International Port Security Program seeks to change how overseas ports are managed, to mitigate security threats to American shipping and combat terrorism and the proliferation of weapons of mass destruction. The USCG has developed extensive rules, processes and institutional templates that it disseminates to overseas territories, supporting their enactment through capacity-building measures, and even inspecting implementation in practice.[34]

[30] Anne-Marie Slaughter, *A new world order* (Princeton: Princeton University Press, 2004); Ian Goldin, *Divided nations: why global governance is failing and what we can do about it* (Oxford: Oxford University Press, 2013).
[31] Hameiri and Jones, 'Global governance', p. 799.
[32] Hameiri and Jones, *Governing borderless threats*.
[33] J. C. Sharman, *The money laundry: regulating criminal finance in the global economy* (Ithaca, NY: Cornell University Press, 2011).
[34] US Coast Guard, *International port security programs*, Department of Homeland Security, n.d., https://www.dco.uscg.mil/Our-Organization/Assistant-Commandant-for-Prevention-Policy-CG-5P/International-Domestic-Port-Assessment.

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The third element of GGST is the use of IOs for meta-governance. In the regulatory state, meta-governance—the governance of governance—is now the core function of most central government agencies. They rarely develop capacities and intervene directly to solve social and political problems, but instead describe broad guidelines and regulations designed to steer a fragmented array of quasi-independent public, private and hybrid actors.[35] IOs' core function is also increasingly meta-governance. They seldom govern issues directly or enforce global rules on member states; rather, they chiefly operate through meta-governance, seeking to cultivate changes *within* states to harmonize domestic policies, institutions and procedures. They are often the institutional spaces in which the aforementioned transnationalized agencies generate guidance and regulations for domestic agencies to enact. IOs are also frequently involved in capacity-building and benchmarking efforts. For example, FATF-style regional bodies coordinate anti-money-laundering training and peer review exercises, while the IMF monitors compliance as part of its economic surveillance function. Similar processes occur in global health, as we shall see.

The pathologies of global governance as state transformation

Superficially, GGST has often circumvented deadlock in traditional multilateral cooperation, generating numerous agreements, guidelines and blueprints, and associated implementation and benchmarking efforts. However, GGST suffers from serious pathologies, stemming from two key features of this neo-liberal approach to problem-solving: the underlying motivation to depoliticize and de-democratize policy-making; and the contradiction between the dependence on domestic state apparatuses for implementation, on the one hand, and the continued hollowing out of state and IO capacities, on the other.

First, the attempt to denationalize and depoliticize policy-making in favour of transnational technocratic problem-solving compounds a democratic deficit that undermines GGST's legitimacy and efficacy. Increasingly, decisions affecting ordinary citizens are made in remote, inter-elite settings closed to public participation and oversight. This compounds the de-democratizing effects of neo-liberal state transformation, exacerbating the 'void' that has opened up between national elites and the citizens they ostensibly represent.[36] While this was once manifest in 'IMF riots' in developing countries, today populists oppose transnationalized elite governance in developed countries, most notably in the EU, its most advanced exemplar.[37]

This is not simply a normative critique. Delinking policy-making from domestic political processes tends to weaken the crucial domestic implementation of GGST

[35] Jonna Gjaltema, Robbert Biesbroek and Katrien Termeer, 'From government to governance … to meta-governance: a systematic literature review', *Public Management Review* 22: 12, 2019, pp. 1760–80.
[36] Peter Mair, *Ruling the void: the hollowing of western democracy* (London: Verso, 2013).
[37] Joachim Blatter and Johannes Schulz, 'Intergovernmentalism and the crisis of representative democracy: the case for creating a system of horizontally expanded and overlapping national democracies', *European Journal of International Relations* 28: 3, 2022, pp. 722–47.

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regimes. Policies emerging from domestic political contestation have stronger foundations than those developed purely through elite interstate dialogue. First, they are likelier to reflect the demands of domestic interest groups that are motivated to ensure they are enacted and punish leaders who fail to deliver. Second, they are more likely to have been discussed publicly, resulting in debate over the issues and prospective solutions and trade-offs, producing wider commitment to chosen policies, with corresponding implications for resourcing and institutional change.

The second, related pathology is that while GGST depends on domestic state apparatuses to enact global rules, under neo-liberalism substantive state capacities have been hollowed out. The transition to the regulatory state has entailed the widespread privatization and outsourcing of state functions and the shift of central governments towards meta-governance. While the state's 'rolling back' in the 1980s has morphed into the 'rolling out' of market-promoting institutions, this means the expansion of regulatory apparatuses, not functional bodies tasked with directly solving social and economic problems.[38] Consequently, central governments are often merely involved in 'cascading' regulatory meta-governance from international contexts into their jurisdictions by developing national guidelines, regulations and so on, while substantive implementation is left to other governance actors. However, these actors are often overloaded, under-resourced, or simply uninterested in implementing global mandates, their operations shaped by more immediate and powerful considerations.[39]

This reflects the fact that states are never neutral instruments of governance but are always conditioned by socio-political struggles embedded in broader social power relations.[40] Efforts to refashion state institutions, even in the name of apolitical problem-solving, will often generate winners and losers, resulting in struggles between powerful groups determining how much transformation occurs and to whose benefit. This often produces a yawning gap between the ostensible goals of GGST and the quality of substantive implementation.[41]

Even where domestic contestation is absent, implementation is often lacklustre, tending to follow the grain of existing power relations and agendas. Elites and officials whose true priorities lie elsewhere may pursue 'mock compliance' designed to satisfy international monitoring processes but without substantively changing domestic governance.[42] This is facilitated by GGST's technocratic benchmarking processes, which frequently measure compliance through light-touch auditing, often relying on states' own self-reporting. As in many contemporary regulatory contexts, including state efforts to regulate the private sector, this can result in a 'box-ticking' approach where audit requirements are notionally satisfied but without meaningful changes occurring.[43]

[38] Jamie Peck and Nik Theodore, 'Still neo-liberalism?', *South Atlantic Quarterly* 118: 2, 2019, pp. 245–65.
[39] Hameiri and Jones, *Governing borderless threats*.
[40] Bob Jessop, *State power: a strategic–relational approach* (Cambridge: Polity, 2008).
[41] Hameiri and Jones, *Governing borderless threats*.
[42] Andrew Walter, *Governing finance: east Asia's adoption of international standards* (Ithaca, NY: Cornell University Press, 2008).
[43] Lee Jones and Shahar Hameiri, 'COVID-19 and the failure of the neo-liberal regulatory state', *Review of International Political Economy* 29: 4, 2022, p. 1034.

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Finally, weaknesses in state capacity are compounded by powerful states' hobbling of IOs. While in the postwar era IOs played a modest, supportive role in building nation-states' capacity and autonomy, since the 1980s they have been starved of resources and subordinated to powerful donor states' and private donors' interests. Consequently, they lack the capacity not only to manage issues directly but also to help states to develop the capabilities needed to implement GGST regimes. In crisis situations, IOs must often appeal to donors for ad hoc funding, while direct action often depends on deploying wealthy states' own capacities. As the following sections show, these pathologies are all visible in GHG.

The WHO: from international organization to meta-governance

In this section, we will first demonstrate how the WHO and its efforts to manage pandemics have been affected by the shift from international organization to GGST. In the early postwar decades, especially with the rise of Third World activism, the WHO was increasingly involved in building states' domestic health-care capacity. However, following the neo-liberal revolt, it was placed under donor control, its agenda giving way to pro-market meta-governance. This has compromised the WHO's independence and its pandemic response capacity, leaving it unable to do much beyond regulating and evaluating national arrangements. With national health-care systems increasingly hollowed out in the neo-liberal era, long before COVID-19 pandemic containment became dangerously dependent on ad hoc efforts by powerful states.

The WHO's transformation reflects the broad shifts described above. In the early postwar decades, international health cooperation was part of the wider drive to support decolonizing countries' national 'modernization'.[44] After an initial focus on disease eradication campaigns, under Soviet and Third World pressure, the WHO became involved in building up postcolonial states' health systems.[45] This process peaked in the 1970s when communist and Third World governments advanced a 'health for all' agenda as part of the NIEO moment, framing health as a human right and a socio-economic issue. Although the WHO secretariat diluted their demands to appease developed countries, majoritarian institutional arrangements and compulsory financial contributions allowed this agenda to take hold, as reflected in the 1978 Alma Ata Declaration.[46]

However, in the subsequent neo-liberal counter-revolution, the WHO was hollowed out and public health care was eroded in favour of marketized provision. In 1982 the WHO budget was frozen and in 1985 the United States withheld

[44] Sunil S. Amrith, *Decolonizing international health: India and southeast Asia, 1930–65* (Basingstoke: Palgrave Macmillan, 2006), p. 72.

[45] Socrates Litsios, 'The long and difficult road to Alma-Ata: a personal reflection', *International Journal of Health Services* 32: 4, 2002, pp. 709–32.

[46] Nitsan Chorev, *The World Health Organization between North and South* (Ithaca, NY, and London: Cornell University Press, 2012); Litsios, 'The long and difficult road'; Theodore M. Brown, Marcos Cueto and Elizabeth Fee, 'The World Health Organization and the transition from "international" to "global" health', in Alison Bashford, ed., *Medicine at the border: disease, globalization and security, 1850 to the present* (Basingstoke: Palgrave Macmillan, 2007), p. 84.

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80 per cent of its financial commitment in protest at the WHO's Essential Drugs Program, which American pharmaceutical companies opposed.[47] Wealthy states increasingly rerouted support for health-care development through the World Bank, thus subordinating it to the structural adjustment agenda, leading to a wave of privatizations and cost-cutting, followed by a push for public–private partnerships.[48] This was paralleled in the North by the marketization of public provision, privatization, public–private partnerships and, especially after the 2008 financial crisis, austerity,[49] often promoted by transnational governance regimes devoted to market deepening, such as the World Trade Organization and the EU.

The WHO's operational budget was slashed, leaving it dependent on voluntary donations. By the 2000s, discretionary donor funding comprised around 80 per cent of WHO expenditure, making it subservient to wealthy countries' demands.[50] Its work was thereby redirected from broad-based support for public health capacity-building towards narrowly specified interventions, typically focused on containing communicable diseases that Northern states feared would spread to their countries.[51] The WHO is also forced to share the GHG space with (and seek funding from) newly empowered private actors, such the Bill and Melinda Gates Foundation, which are independent from democratic oversight but exercise enormous influence over global health policy—routinely promoting privatized solutions.[52]

Budgetary constraints forced the WHO to embrace meta-governance as its main *modus operandi*. This began in the late 1990s with its Influenza Pandemic Plan blueprint, which provided a template for member states to follow when devising their own national policies. The approach accelerated as the WHO latched on to the emerging 'global health security' agenda, which reflected the attempt of Northern states' public health agencies to find new purpose and defend their resources from neo-liberal cuts by presenting 'emerging diseases' in developing countries as threats to their own countries' security.[53]

The WHO responded by developing the revised IHR in the wake of the 2002–2003 SARS outbreak. The original 1969 IHR had reflected the 'Westphalian' norms of the time, requiring states to report outbreaks of only six communicable diseases and focusing on key gateways such as ports and airports. The Chinese government's uncooperative approach during SARS fuelled calls for reform. The new IHR, developed by internationally networked medical and scientific officials, marked a decisive shift towards GGST. They specified extensive governance and policy changes that WHO member states should enact domestically to prepare for and contain epidemics.

[47] Brown et al., 'The World Health Organization', p. 85.
[48] Brown et al., 'The World Health Organization', p. 85.
[49] Fran Collyer and Kevin White, 'The privatization of Medicare and the National Health Service, and the global marketization of healthcare systems', *Health Sociology Review* 20: 3, 2011, pp. 238–44.
[50] Sara E. Davies, *Global politics of health* (Cambridge: Polity, 2010), pp. 34–5.
[51] Simon Rushton, 'Global health security: security for whom? Security from what?', *Political Studies* 59: 4, 2011, pp. 779–96.
[52] Sophie Harman, 'The Bill and Melinda Gates Foundation and legitimacy in global health governance', *Global Governance* 22: 3, 2016, pp. 349–68; see also Simon Rushton and Owain David Williams, eds, *Partnerships and foundations in global health governance* (Basingstoke: Palgrave Macmillan, 2011).
[53] WHO, *The World Health Report 2007—a safer future: a global public health security in the 21st century* (Geneva, 2007).

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Crucially, the WHO acquired no supranational enforcement powers. Critics are therefore correct in saying that states did not sacrifice their sovereignty to the WHO, but this is not how GGST is *meant* to work. The 2005 IHR only mildly enhanced the WHO's authority, allowing it to declare a PHEIC after consulting an expert panel. But this was only intended to spur states to activate their own pandemic preparedness plans. The WHO's main role was to be a meta-governor, defining scientific 'best practice' through the IHR and associated guidance (including specific 'temporary recommendations' during pandemics), then monitoring compliance through benchmarking using numerical indicators.[54] The WHO's surveillance mechanisms relied on voluntary information-sharing by state and non-state participants. During outbreaks, the WHO behaved not as a 'firefighter', rushing to contain disease with its own resources, but rather as an expert coordinator of a fragmented array of governmental, non-governmental and intergovernmental agencies.

Typically for GGST, the IHR's implementation was also to be achieved cheaply, and this had disastrous consequences. The IHR were unaccompanied by additional resources for the WHO to intervene directly in health crises, or to support member states' domestic implementation. Effectively, poorer states—where new diseases were thought to emerge—were urged to rework their domestic health-care systems to contain pandemics that might affect wealthier countries, with the latter offering inadequate support. Moreover, in many developing countries, this agenda was largely divorced from domestic political processes and priorities, resulting in weak implementation even where some international assistance was provided.

Consider, for example, the H5N1 bird flu epidemic, which some epidemiologists claimed could kill between 50 million and 350 million people if the virus mutated to enable human-to-human transmission. The WHO warned of potential economic costs of US$800 billion, but in 2006–2007 donors pledged just US$2.7 billion to manage this threat; and only 72 per cent of this was delivered by the end of 2008, of which just 36 per cent was allocated to support country programmes (over 40 per cent in the form of loans), resulting in an estimated financing gap of 38 per cent, or US$836 million.[55] Reflecting the operation of GGST, donor interventions focused on translating guidelines from the WHO, the Food and Agriculture Organization and the Office International des Epizooties into domestic settings, 'with 109 country plans completed (if not tested) by the end of 2007'.[56]

Behind this apparent achievement, however, the inadequate resourcing of these interventions and their disconnection from domestic political processes meant that implementation remained illusory. The centre of the epidemic, Indonesia, with a population of over 242 million, received just US$132 million, which, as elsewhere, was used to reconfigure animal and human health systems around international

[54] Hameiri and Jones, *Governing borderless threats*, pp. 131–4.

[55] UN System Influenza Coordinator and World Bank, *Responses to avian influenza and state of pandemic readiness: fourth global progress report* (New York: UN, 2008), pp. 13–20.

[56] Ian Scoones and Paul Forster, *The international response to highly pathogenic avian influenza: science, policy and politics* (London: Earthscan, 2008), p. 8.

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guidelines.[57] However, local authorities' commitment was weak, with just US$57 million or 1.7 per cent of the national health budget devoted to bird flu. Officials explained that, despite wealthy countries' focus on the disease, it was just one of many health challenges confronting Indonesia, and far from the deadliest.[58] With little domestic clamour to prioritize bird flu, there was insufficient democratic pressure to enact serious governance reforms. Consequently, powerful poultry-producing firms—the disease's real epicentre—deflected donor and government initiatives towards 'backyard' farmers, undermining containment efforts.[59]

The illusory quality of GGST did not improve much thereafter. The 'lessons learned' from the 2009 swine flu and 2014 Ebola outbreaks were practically identical, emphasizing states' lack of basic public health capacity.[60] By 2018, the WHO's Global Preparedness Monitoring Board estimated that only 'one-third of countries [had] the capacities required' by the IHR (see Table 2).[61] As Scoones and Forster observed, the 'carefully produced booklets and presentations' surrounding the implementation of international standards are 'open to … not a little derision … When asked what he would do if a pandemic occurred, a senior official at the WHO answered succinctly, "head for the hills". It was not a joke.'[62]

Table 2: States' compliance with WHO IHR core capacity requirements, 2017 (percentage at each level of implementation)

	Full (100%)	High (75–99%)	Moderate (50–74%)	Low / Missing (<50%)	Distribution
Chemical	12	22	14	52	
Coordination	36	15	13	35	
Food safety	29	21	19	30	
Human resources	24	8	17	51	
Laboratory	22	34	22	22	
Legislation	44	0	13	43	
Points of entry	8	30	12	50	
Preparedness	24	19	20	37	
Radionuclear	19	18	15	47	
Response	19	37	17	27	

Source: WHO, 'All capacities—data by country', 2021, https://apps.who.int/gho/data/node.main.IHR00ALLN?lang=en. *Note*: 16 % of data missing for each category. IHR required full compliance by 2012.

[57] UN System Influenza Coordinator and World Bank, *Responses*, pp. 13–20.
[58] Hameiri and Jones, *Governing borderless threats*, pp. 147–8.
[59] Brian Perry, Kamarudin Md. Isa and Carlos Tarazona, *Independent evaluation of FAO's participatory disease surveillance and response programme in Indonesia* (Rome: FAO, 2009).
[60] Trygve Ottersen, Steven J. Hoffman and Gaëlle Groux, 'Ebola again shows the International Health Regulations are broken: what can be done differently to prepare for the next epidemic?', *American Journal of Law and Medicine* 42: 2–3, 2016, pp. 356–92.
[61] Global Preparedness Monitoring Board, *A world at risk: annual report on global preparedness for health emergencies* (Geneva: WHO, 2019), p. 20.
[62] Scoones and Forster, *The international response*, p. 36.

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Consequently, in practice, containing epidemics relied less on sophisticated meta-governance than on reactive, ad hoc emergency responses by powerful states, as seen with bird flu, swine flu and Ebola. In each case, reflecting its hollowing out by donors, the cash-strapped WHO cited catastrophic projections of potential deaths to galvanize a response from wealthy countries.[63] This became highly controversial during swine flu, when the WHO was accused of deliberately exaggerating the disease's severity.[64] When Ebola broke out, the WHO was especially underfunded as its emergency response budget had been halved in 2013 to just US$228 million.[65] However, fearing a repeat of the criticism levelled at it, the WHO hesitated, failing to challenge African states' under-reporting and not declaring a PHEIC until four months after the first confirmed cases.[66] Ultimately, the UN Security Council took up the crisis, leading to medical and even military deployments from China, the United States and other western countries to control the disease.[67] But such interventions, all short-term and sometimes ending before outbreaks were even contained, did little to build long-term domestic health-care capacity. Although the Ebola fiasco also prompted the formation of a WHO global health emergencies task force, this could not compensate for GGST's weaknesses.

COVID-19 and the failure of global health governance

GGST's pathologies, visible for years previously, were grievously exposed when COVID-19 struck, but with two important differences from previous episodes. First, the hollowing out of the WHO left it subservient to major donors, but these now included China, where the outbreak originated, meaning its response was even more dangerously hobbled. Second, the IHR relied on domestic implementation, but COVID-19 showed this was a problem for developed countries as much as developing ones, causing the swift collapse of GHG and limiting the emergency mobilizations that had contained previous epidemics.

COVID-19 clearly exposed the WHO's hollowing out and subservience to major donors, but with one major change since the SARS outbreaks: China's emergence as a major donor. During SARS, the WHO openly criticized Chinese officials' conduct. During COVID-19, however, the WHO was practically sycophantic. Through December 2019 and January 2020, local Chinese officials were still suppressing information and arresting medical whistle-blowers, and by mid-January Beijing was insisting that the outbreak was under control, no evidence of human-to-human transmission existed, and travel restrictions were unnecessary. Although privately WHO officials were increasingly frustrated by Chinese stonewalling, publicly they praised China's 'transparent' conduct as 'very impressive',

[63] Kavanagh et al., 'Playing politics', p. 37.
[64] Donald G. McNeil, 'Response of WHO to swine flu is criticized', *New York Times*, 11 March 2011, https://www.nytimes.com/2011/03/11/health/policy/11flu.html.
[65] Declan Butler, 'Agency gets a grip on budget', *Nature* 498: 7452, 2013, pp. 18–19.
[66] Adam Kamradt-Scott, 'WHO's to blame? The World Health Organization and the 2014 Ebola outbreak in West Africa', *Third World Quarterly* 37: 3, 2016, pp. 401–18 at pp. 404, 409–10.
[67] Kamradt-Scott, 'WHO's to blame?'.

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'incredible', 'beyond words' and setting 'a new standard for outbreak response'.[68] The WHO endorsed Beijing's misleading claims about community transmission on 14 January, changing its line only after Chinese officials changed theirs on 22 January.[69] It recommended airport screening only after the 23 January Wuhan lockdown, and continued supporting Beijing's opposition to border closures until 29 February, long after they would have helped to contain the virus.[70]

The WHO's most disastrous act was to endorse China's lockdown policy. Following a visit by officials in late February 2020, the WHO praised 'China's bold approach', claiming it had 'changed the course of a pandemic' and urging other states to take 'urgent and aggressive actions' modelled on Beijing's policy.[71] By this time, aided by asymptomatic transmission, SARS-CoV-2 had already spread to 48 countries, making a SARS-like containment effort 'doomed from the outset'.[72] The WHO's endorsement of lockdown was especially remarkable given that it had never featured in the IHR or any WHO guidance. This is because, as subsequent modelling suggests, lockdowns cause far greater harm to public health than they prevent.[73] Associated policies such as rigorous contact-tracing, which the WHO also promoted, had also been rejected in the WHO's most recent (December 2019) guidance.[74] Thus, when WHO supporters claim that its 'biggest impediment' was 'the failure of its member states to comply fully with the IHR',[75] they ignore the organization's disregard for its *own* rules as it rushed to praise China. Its endorsement of lockdown fuelled mounting panic and calls for extraordinary measures across the world, creating intense media and public pressure on other governments to follow suit. It continued supporting lockdowns long after it became clear that eradication was impossible, when 'it clearly wasn't working and was causing immense harm', moderating its advice only in October 2020, when WHO's COVID-19 special envoy David Nabarro told member states to 'stop using lockdown as your primary control method'.[76]

However, the WHO's servility to China was the natural result of its hollowing out by powerful states. President Trump was the WHO's most prominent critic, freezing US funding in April 2020 and later declaring that Washington would withdraw from the WHO entirely—a policy President Biden subsequently reversed. But Japan's deputy prime minister also blasted the 'Chinese Health Organization', the Australian government demanded an investigation into the

[68] Associated Press, 'China delayed releasing coronavirus info, frustrating WHO', AP News, 2 June 2020, https://apnews.com/3c061794970661042b18d5aeaaed9fae; Emily Rauhala, 'Chinese officials note serious problems in coronavirus response: the World Health Organization keeps praising them', *Washington Post*, 8 Feb. 2020, https://www.washingtonpost.com/world/asia_pacific/chinese-officials-note-serious-problems-in-coronavirus-response-the-world-health-organization-keeps-praising-them/2020/02/08/b663dd7c-4834-11ea-91ab-ce439aa5c7c1_story.html.

[69] Lilleker and Gregor, 'World Health Organisation', p. 28.

[70] Mark Woolhouse, *The year the world went mad* (Muir of Ord: Sandstone, 2022), p. 221.

[71] Woolhouse, *The year the world went mad*, pp. 19–20.

[72] Woolhouse, *The year the world went mad*, pp. 52–3.

[73] See e.g. Jonas Herby, Lars Jonung and Steve Hanke, 'A literature review and meta-analysis of the effects of lockdowns on COVID-19 mortality', *Studies in Applied Economics*, no. 210, May 2022.

[74] Toby Green, *The Covid consensus: the new politics of global inequality* (London: Hurst, 2021), pp. 174–7.

[75] Patrick, 'When the system fails', p. 47.

[76] Woolhouse, *The year the world went mad*, p. 54.

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coronavirus's origins and the WHO's conduct, and in May 2020 140 states backed an EU-sponsored World Health Assembly motion to establish an inquiry into the management of the pandemic. Although wealthy states were right to criticize the WHO's deference to China, they were blaming the organization for weaknesses created by their own policies. As noted earlier, they had deliberately made the WHO dependent on voluntary funding to neutralize the 'health for all' agenda. In 2019/20, its total budget was just US$4.8 billion, smaller than that of many American hospitals, and under 40 per cent of that of the US Centers for Disease Control.[77] Its emergency response budget was a mere US$154 million in 2018/19.[78]

In these circumstances, alienating Beijing would have been irrational. While historically China had provided little voluntary funding to the WHO, by 2017 it was the third largest compulsory financial contributor,[79] with huge potential to contribute more voluntarily. Indeed, in May 2020 China pledged US$2 billion over two years to the WHO's COVID-19 budget, which stood to make it the organization's top funder. When coupled with the fact that the WHO's disease surveillance programme depends on member-state cooperation, it is clear that the organization had been predisposed to servility to Beijing.[80] Those lamenting the absence of US leadership during COVID-19 ignore the earlier impact of such 'leadership' in hollowing out the WHO.

The pandemic also exposed GGST's second key pathology: its reliance on hollowed-out states. This time, however, the problem was revealed in the world's wealthiest and ostensibly best-governed societies. Indeed, COVID-19 showed that domestic implementation was even worse than previously understood. The compliance data presented in Table 2, based on states' self-reporting, indicate that the world's wealthiest states were best prepared. Similar technocratic benchmarking exercises, including those conducted by the World Economic Forum and leading scientists, concurred.[81] The illusory quality of GHG was quickly exposed, however, as these states could not cope with the onset of a serious pandemic. The US and the UK, for example, were consistently ranked as best-prepared, but had some of the world's worst infection and mortality outcomes in 2020.

These results stemmed from these states' neo-liberal transformation, which stripped them of substantive capacities. They were left reliant on hollow regulatory planning and dependent on often inept private-sector actors, underscoring how GGST regimes' domestic implementation is shaped by social power relations. For example, the IHR (annex 1) commit states to providing 'support through

[77] Kavanagh et al., 'Playing politics', p. 37; Office of Financial Resources, *FY 2019 snapshot* (Washington DC: US Centers for Disease Control and Prevention, 2019), https://www.cdc.gov/funding/documents/fy2019/fy-2019-ofr-snapshot-508.pdf.

[78] WHO, *Programme budget, 2018–19* (Geneva, 2017), p. 9.

[79] WHO, *Assessed contributions: 2018–19*, World Health Organization, 2017, http://open.who.int/2018-19/contributors/overview/ac.

[80] Kavanagh et al., 'Playing politics', pp. 42–3.

[81] World Economic Forum, 'These are the top 10 countries for pandemic preparedness', 15 Nov. 2019, https://www.weforum.org/agenda/2019/11/countries-preparedness-pandemics; Ben Oppenheim, Mark Gallivan, Nita K. Madhav, Naor Brown, Volodymyr Serhiyenko, Nathan D. Wolfe and Patrick Ayscue, 'Assessing global preparedness for the next pandemic: development and application of an epidemic preparedness index', *BMJ Global Health* 4: 1, 2019, e001157.

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specialized staff, laboratory analysis of samples … and logistical assistance (e.g. equipment, supplies and transport)'.[82] In Britain, however, successive reforms had actually reduced relevant capacity, with the number of public health laboratories declining from 60 in the 1970s to nine by 2012. Under post-2011 austerity, operational responsibility for public health had been devolved to local governments, whose funding was cut by £700 million in real terms between 2015 and 2019.[83] Consequently, by mid-February 2020, the UK could only conduct five COVID-19 tests per week.[84] As for 'logistical assistance', the privatization of the National Health Service's logistics left the UK reliant on private intermediaries operating 'just-in-time' business models, resulting in massive shortages of key equipment.[85] The National Audit Office belatedly reported that this 'operating model was not designed to respond to a pandemic', resulting in a desperate, competitive scramble for supplies from overseas.[86] Even the stockpile of equipment for use during a pandemic had been outsourced, to a foreign multinational whose warehouses were understocked by 10–28 per cent, with 45 per cent of personal protective equipment (PPE) having expired on the shelves. Health-care staff had to ration PPE and improvise, using items that even included garbage bags.[87] The sort of dysfunction previously seen as exclusive to developing countries thus erupted within the world's richest states, sending governments panicking into lockdown.

The yawning gap between these states' ostensible and real pandemic preparedness owes much to regulatory statehood and the technocratic, de-democratized nature of GGST. On paper, western states had been preparing since the late 1990s, with technocrats faithfully translating WHO guidelines into domestic plans and placing pandemic disease high up on their governments' risk registers. But, reflecting the nature of the regulatory state, they rarely developed substantive capabilities to contain new diseases, not least because plans were rarely accompanied by additional funding or capacity-building. Agencies cascaded WHO guidance down into national plans, intended to spur the development of more detailed planning by others, but in many cases this simply generated additional plans on paper, satisfying external audits but changing little substantively. When COVID-19 struck, the regulatory state was exposed as regulation all the way down.

This outcome epitomizes a technocratic approach that marginalizes public deliberation and involvement to the detriment of meaningful implementation.

[82] WHO, *International Health Regulations (2005)*, 3rd edition (Geneva, 2016), https://www.who.int/publications/i/item/9789241580496, p. 41.

[83] Jones and Hameiri, 'COVID-19', pp. 1037–8.

[84] Laura Donnelly and Tom Morgan, 'UK abandoned testing because system "could only cope with five coronavirus cases a week"', *Telegraph*, 30 May 2020, https://www.telegraph.co.uk/news/2020/05/30/revealed-test-trace-abandoned-system-could-cope-five-coronavirus.

[85] David Hall, John Lister, Cat Hobbs, Pascale Robinson and Chris Jarvis, 'Privatised and unprepared: the NHS supply chain', University of Greenwich/We Own It, 2020, https://weownit.org.uk/privatised-and-unprepared-nhs-supply-chain.

[86] National Audit Office, *Investigation into government procurement during the COVID-19 pandemic* (London, 2020), https://www.nao.org.uk/report/government-procurement-during-the-covid-19-pandemic, p. 6.

[87] Hall et al., 'Privatised and unprepared', p. 20; 'UK failed to stockpile crucial PPE', BBC News, 28 April 2020, https://www.bbc.com/news/newsbeat-52440641.

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Consider the UK's pandemic preparedness strategy which, appealing to scientific best practice, adopted a 'business-as-usual' approach entailing no additional capacity-building and few changes to everyday life, directing planners to deal with 210,000–315,000 excess deaths over a 15-week period and envisioning health-care rationing according to a 'checklist' prepared by ethical experts.[88] It may well be true, as the strategy asserted, that it is impossible to stop a new pandemic disease, and that governments could at best mitigate the spread and its impact. However, this case had never been made to the country's citizens. Despite pandemics being designated a 'tier one' security threat, their management had never featured significantly in the UK's public debate or democratic politics. Had it done so, consent might have been built for a 'business-as-usual' approach; or the policy might have been changed, with greater public scrutiny entailing a more serious devotion of resources, capacity-building and effort. Indeed, in several east Asian states, poor performance during earlier epidemics resulted in the reversal of some debilitating neo-liberal reforms and greater capacity-building.[89] However, in the UK, when SARS-CoV-2 struck, despite decades of elite efforts to lower public expectations of the state, many citizens rejected the 'business-as-usual' approach, demanding tougher government action. Lacking the capacity for any alternative, however, the UK government collapsed repeatedly into lockdown. As a member of the UK Scientific Advisory Group on Emergencies observed,

> from a government perspective, lockdown had big advantages: it didn't require any forward planning, there was no need to build capacity in advance, and no direct financial cost. All lockdown took was a government decree and a modicum of enforcement. It was a lazy solution … as well as a hugely damaging one. Avoiding lockdown would have required a lot more effort.[90]

Neo-liberal state failure in the system's core stymied the ad hoc emergency interventions that had kept GHG afloat in previous crises. Whereas previously Northern states had sent troops to aid the global South, now they deployed one of their last few functional state apparatuses internally, to build hospitals and provide emergency medical and logistical support. Their lack of preparedness and dependence on outsourcing and fragile global supply chains led to a mad scramble for equipment and vaccines, entailing competitive and even zero-sum behaviour. Confronting angry and fearful citizens, governments survived by catering first to them, through ad hoc innovations that disregarded the IHR and often WHO guidance. While the WHO struggled to maintain some semblance of international solidarity, its programmes remained systematically underfinanced, with just one-third of requested funds being secured for 2021–22.[91]

[88] UK Department of Health, *UK influenza pandemic preparedness strategy* (London, 2011), https://assets.publishing.service.gov.uk/government/uploads/system/uploads/attachment_data/file/213717/dh_131040.pdf, p. 62.
[89] Jones and Hameiri, 'COVID-19', pp. 1048–9.
[90] Woolhouse, *The year the world went mad*, p. 255.
[91] WHO, *Access to COVID-19 tools funding commitment tracker*, 2022, https://www.who.int/publications/m/item/access-to-covid-19-tools-tracker.

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Conclusion

Contributing to the emerging literature on the failure of GHG during COVID-19, this article has argued that this outcome was predictable, given the pathologies of GGST. Arising from the neo-liberal counter-revolution of the 1980s, GGST seeks to address global issues cheaply and in a technocratic, de-democratized fashion, erecting global rules and regulations then spurring states to implement them domestically. But the hollowing-out of IOs and states alike under neo-liberalism has undercut this approach, since it entails a failure to build real capacities where they matter most: on the ground. Whereas previous epidemics only exposed this problem in developing countries, with COVID-19 the same pathologies were exposed in the global North, leading to GHG's near-total breakdown.

This argument has important implications for the reform of GHG, and of global governance more broadly. Too much emphasis has been placed on the 'global' at the expense of the 'local' or national. This is reflected in many contemporary reform proposals, which overwhelmingly focus on 'strengthening' the WHO and empowering it to 'enforce' the IHR on recalcitrant states.[92] Like Wenham and colleagues,[93] we are sceptical about calls for another pandemic treaty, but not because, as they argue, the prospects for a 'globalist, quasi-cosmopolitan approach' are weak, given the dominance of a 'nation-state first' approach. COVID-19 showed that the 'globalist' approach is illusory and that hostility to the nation-state is deeply problematic. The resilience of 'global' health governance depends overwhelmingly on what happens at the national and subnational levels. Policy-makers have been building castles in the sky when they should have been building ramparts on the ground.

This is an argument not for retreating into selfish nationalism, but for greater realism in our evaluation and pursuit of global governance. The COVID-19 pandemic teaches us that to solve the problems of global health—and arguably other problems confronting humankind, such as climate change—no amount of global treaties, regulations and guidelines can take the place of well-organized, well-resourced and responsive state institutions. Equally, the pandemic revealed that the previous decades' neo-liberal reforms have failed to deliver such institutions—in fact, that they have had quite the opposite effect. To have any prospects for success, GHG will need to be rebuilt from the bottom up.

Northern states need to get their own houses in order and invest in meaningful pandemic preparedness and response capacities. Success will require radical changes in domestic governance, particularly far stronger democratic accountability and mobilization to push political leaders to abandon failed neo-liberal, regulatory governance and provide necessary public goods directly. Northern states should also provide genuine support and resources to help developing countries strengthen their own health systems. Rising tensions between the United States and its allies and China might benefit poorer countries here. Beijing made much capital from its COVID-19 'health diplomacy', and Northern states will not want

[92] See e.g. Gostin, 'COVID-19 reveals'; Taylor and Habibi, 'The collapse'.
[93] Wenham et al., 'The futility', pp. 838–9.

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to abandon the field to China. Neither side seems likely to resource the WHO to build developing countries' health-care capacity directly. But the organization's role could extend to helping governments identify gaps, providing information which poorer states could use when negotiating with richer states for funding. The WHO's assessments should also be tailored to the characteristics and needs of different societies and states, rather than to measuring whether they meet the technocratic benchmarks of regulatory meta-governance—which has systematically failed us all.

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